



P35 · 50 PAGES, PRINTABLE + FILLABLE, ONE FULL CYCLE

An IVF Cycle Planner and Journal

A fifty-page planner for one IVF cycle. Pre-cycle team and cost setup, the daily stim-phase tracker, monitoring-appointment log, retrieval day page, fertilization and blastocyst report tracker, transfer day or freeze-all decision page, two-week wait, beta test day, and the pages for whichever way the cycle lands. Medications are tracked by purpose, not by name. The planner is the record. The decisions stay with you and your reproductive endocrinologist.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

A note before you start

You are about to do one of the hardest logistical things a person can do.

Injections at specific times. Multiple appointments per week. Bloodwork numbers that arrive by app or by phone. Follicle counts. Embryo grades. A transfer day, or a freeze-all decision. A two-week wait that feels longer than the previous ten days combined. A beta test that tells you the next chapter.

The clinic has the data. You have the days. This planner is for the days.

It is not a medical book. It does not tell you which protocol to use. It does not name any medication. It is the place to write down what your clinic prescribed (by purpose, in your own words), what your numbers were today, what the nurse said on the phone, what you are scared of, what you are angry about, and what you want to remember when this cycle is behind you.

You can use the planner for one cycle. If there is a next cycle, you re-print.

Soothemade

This is a planning and tracking tool, not medical advice. For specific medical questions about your IVF cycle, please contact your reproductive endocrinologist or clinical team. We do not name medications, prescribe a protocol, or predict outcomes. The clinical decisions in this cycle belong to you and your provider.

How to use this planner

- Print the whole planner, or only the pages you need for this cycle.
- Carry the current week's pages with you to monitoring appointments.
- Fill in the daily tracker each evening, or the next morning. The hardest cycles to remember are the ones you do not write down in real time.
- The "telling people / not telling people" page is at the back. Read it before you tell, or before you do not tell. There is no wrong answer.
- The two financial pages, pre-cycle costs and running tally, are not optional. The financial weight is real. Looking at it directly is one of the things this planner does that the clinic's patient portal does not.

Your team

The numbers and people you will reach for during this cycle.

REPRODUCTIVE ENDOCRINOLOGIST

Name: _____
Direct number: _____
Clinic main number: _____
After-hours line: _____

PRIMARY NURSE / CARE COORDINATOR

Name: _____
Number: _____
Best time to reach them: _____

PHARMACY

Name: _____
Number: _____
Hours: _____

INSURANCE / FINANCIAL CONTACT

Insurance company: _____
Member ID: _____
Pre-authorization number: _____
Financial counselor name: _____
Counselor number: _____

EMERGENCY (NOT THE CLINIC)

Nearest ER: _____

When to go to the ER:

- Severe pelvic pain that is new
- Heavy bleeding
- Sudden severe nausea or vomiting
- Rapid weight gain over a day or two
- Trouble breathing

Always call the clinic first if you can.

Pre-cycle calendar setup

The dates the clinic has given you, mapped to your life.

This cycle's plan, as I currently understand it:

Baseline / start day: _____
First stim injection day: _____
First monitoring scan: _____
Estimated retrieval window: _____ to _____
Estimated transfer window: _____ to _____
Beta test day (estimated): _____

CONFLICTS I NEED TO SOLVE FOR:

Work / travel: _____

Childcare for older kids: _____
Pet care: _____
Other: _____

WHO IS HELPING WITH WHAT:

Driving me to retrieval: _____
Driving me to transfer: _____
Doing morning injections: me / partner / both
Doing evening injections: me / partner / both
Picking up meds at pharmacy: _____

Pre-cycle cost setup

Looking at the financial weight directly. Filling this in is not optional.

WHAT THE CLINIC TOLD ME THIS CYCLE WILL COST

Cycle base price: \$ _____
Monitoring (if separate): \$ _____
Anesthesia: \$ _____
Embryology lab fees: \$ _____
Cryopreservation / storage: \$ _____
Genetic testing of embryos (if doing): \$ _____
Medications (estimated): \$ _____
Transfer fees (if separate): \$ _____
ICSI or other techniques: \$ _____

ESTIMATED TOTAL FOR THE CYCLE: \$ _____

WHAT INSURANCE IS COVERING

Diagnostic visits: \$ _____
Monitoring: \$ _____
Procedures: \$ _____
Medications: \$ _____

ESTIMATED INSURANCE COVERAGE: \$ _____

OUT-OF-POCKET ESTIMATE

Total cost: \$ _____
minus Insurance: \$ _____
= OUT OF POCKET: \$ _____

HOW I AM FUNDING THIS

- ☐ Savings
- ☐ Family help
- ☐ Loan or credit
- ☐ Employer benefit
- ☐ Grant / financial assistance program
- ☐ Other: _____

What I told myself I would not do to fund this:

The stim phase, what I am tracking

You will inject medications for roughly 8 to 14 days. Your clinic told you which ones and when. This planner does not name them. It tracks them by purpose.

Common categories you may be using (your clinic will tell you what applies to you):

- An **evening stimulation injection**, the daily one that grows follicles
- A morning **stimulation injection**, sometimes a second one a few days in
- A **suppression injection**, sometimes added partway through to prevent early ovulation
- A **trigger shot**, one or two doses, the night before retrieval
- **Luteal support** medications, after transfer, oral or vaginal or injectable

Fill in your specifics with the clinic's exact instructions. Do not work from memory.

MY DAILY MEDICATION PLAN (filled in by ME, after the clinic walked me through it)

Morning:

Time: _____
Type: _____
Route: _____
Notes: _____

Evening:

Time: _____
Type: _____
Route: _____
Notes: _____

Trigger shot, when prescribed:

Time: _____
Notes: _____

Stim day 1

DATE: _____

HOW MANY HOURS OF SLEEP: _____

MORNING INJECTION

Time given: _____

Felt: _____

EVENING INJECTION

Time given: _____

Felt: _____

SIDE EFFECTS NOTICED TODAY

- ☐ Bloating
- ☐ Fatigue
- ☐ Mood swings
- ☐ Headache
- ☐ Tenderness at injection site
- ☐ Nausea
- ☐ Hot flashes
- ☐ Other: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 2

DATE: _____

HOW MANY HOURS OF SLEEP: _____

MORNING INJECTION

Time given: _____

Felt: _____

EVENING INJECTION

Time given: _____

Felt: _____

SIDE EFFECTS NOTICED TODAY

- ☐ Bloating
- ☐ Fatigue
- ☐ Mood swings
- ☐ Headache
- ☐ Tenderness at injection site
- ☐ Nausea
- ☐ Hot flashes
- ☐ Other: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 3

DATE: _____

MORNING INJECTION

Time given: _____

EVENING INJECTION

Time given: _____

SIDE EFFECTS: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 4

DATE: _____

MORNING INJECTION

Time given: _____

EVENING INJECTION

Time given: _____

SIDE EFFECTS: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 5

DATE: _____

MORNING INJECTION

Time given: _____

EVENING INJECTION

Time given: _____

SIDE EFFECTS: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 6

DATE: _____

MORNING INJECTION

Time given: _____

EVENING INJECTION

Time given: _____

SIDE EFFECTS: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 7

DATE: _____

MORNING INJECTION

Time given: _____

EVENING INJECTION

Time given: _____

SIDE EFFECTS: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 8

DATE: _____

MORNING INJECTION

Time given: _____

EVENING INJECTION

Time given: _____

SIDE EFFECTS: _____

ENERGY LEVEL (circle): 1 2 3 4 5

MOOD (one word): _____

ONE LINE ABOUT TODAY:

Stim day 9 + 10 + 11 + 12 (extra)

If your cycle runs longer, use these pages.

DAY ____ DATE: _____

Morning inj. time: _____

Evening inj. time: _____

Side effects: _____

Energy (1-5): ____ Mood: _____

One line:

DAY ____ DATE: _____

Morning inj. time: _____

Evening inj. time: _____

Side effects: _____

Energy (1-5): ____ Mood: _____

One line:

DAY ____ DATE: _____

Morning inj. time: _____

Evening inj. time: _____

Side effects: _____

Energy (1-5): ____ Mood: _____

One line:

DAY ____ DATE: _____

Morning inj. time: _____

Evening inj. time: _____

Side effects: _____

Energy (1-5): ____ Mood: _____

One line:

Monitoring appointment 1

DATE: _____

TIME: _____

BLOODWORK NUMBERS the clinic sent me afterward:

FOLLICLE COUNT (in the clinic's words):

Right ovary: _____

Left ovary: _____

Largest: _____

Lining (if measured): _____

WHAT THE CLINIC TOLD ME TO DO NEXT:

ANY DOSE OR MEDICATION CHANGES (by purpose):

WHAT I FELT WALKING OUT:

Monitoring appointment 2

DATE: _____

BLOODWORK NUMBERS:

FOLLICLE COUNT:

Right: _____

Left: _____

Largest: _____

Lining: _____

NEXT STEP per clinic:

CHANGES:

Monitoring appointment 3

DATE: _____

BLOODWORK NUMBERS:

FOLLICLE COUNT:

Right: _____

Left: _____

Largest: _____

Lining: _____

NEXT STEP per clinic:

Monitoring appointment 4

DATE: _____

BLOODWORK NUMBERS:

FOLLICLE COUNT:

Right: _____

Left: _____

Largest: _____

Lining: _____

NEXT STEP per clinic:

Monitoring appointment 5 (and extras)

DATE: _____

BLOODWORK NUMBERS:

FOLLICLE COUNT:

Right: _____

Left: _____

Largest: _____

Lining: _____

NEXT STEP per clinic:

CHANGES:

The trigger shot

The night-before-retrieval injection (or two). The clinic's instructions on this are extremely precise. Do not improvise.

DATE OF TRIGGER: _____

EXACT TIME the clinic told me to inject: _____

I gave the trigger at: _____

I will arrive at the clinic for retrieval at:

WHO IS DRIVING ME: _____

WHO IS PICKING ME UP: _____

WHAT I HAVE BEEN TOLD ABOUT THE NIGHT BEFORE:

- ☐ Cutoff for eating / drinking: _____
- ☐ What to wear: _____
- ☐ What to bring: _____
- ☐ Where to check in: _____

THE FEELING THE NIGHT BEFORE:

Retrieval day

DATE: _____

ARRIVAL TIME: _____

GOING-IN TIME: _____

WAKE-UP TIME: _____

DISCHARGED: _____

NUMBER OF EGGS RETRIEVED (from the clinic):

WHO WAS WITH ME: _____

HOW I FELT WAKING UP:

WHAT THEY TOLD ME TO DO TONIGHT:

- ☐ Rest, lying down or reclined
- ☐ Hydrate well, including electrolyte drinks if recommended
- ☐ Eat lightly (their guidance)
- ☐ Watch for warning signs (severe pain, heavy bleeding, sudden bloating, trouble breathing)
- ☐ Their specific instructions:

NUMBER TO CALL OVERNIGHT IF SOMETHING FEELS WRONG:

ONE THING I WANT TO REMEMBER ABOUT TODAY:

Fertilization and blastocyst report tracker

The clinic sends a morning report. Day 0 is retrieval. The numbers usually shrink as days go by. This is normal. Not every egg becomes a blastocyst.

DAY 0 (RETRIEVAL DAY)

Eggs retrieved: _____

Mature (M2): _____

DAY 1

Fertilized normally (2PN): _____

Notes from the clinic:

DAY 3

Still developing: _____

Notes:

DAY 5

Blastocysts ready for freeze / transfer:

Embryo grades (in the clinic's words):

DAY 6 (and day 7 if your clinic does that)

Additional blastocysts: _____

Notes:

FINAL EMBRYO COUNT: _____

TESTED (PGT) or UNTESTED: _____

HOW I FELT GETTING EACH DAY'S NUMBER:

Day 1: _____

Day 3: _____

Day 5: _____

Day 6: _____

A NOTE TO MYSELF ABOUT THESE NUMBERS:

The numbers are information, not a verdict. The clinic interprets them.

What I am feeling about them is allowed to be its own thing,
separate from the math.

Transfer day OR freeze-all decision

Some cycles transfer fresh, in the same cycle. Some freeze everything and transfer in a later cycle. Your clinic will recommend based on your situation. Both are common. Neither is wrong.

THIS CYCLE: I am doing (circle one)

- ☐ Fresh transfer
- ☐ Freeze-all (transfer in a later cycle)
- ☐ Mixed (some fresh, some frozen)

REASON the clinic gave for this decision (in their words):

WHAT I AM FEELING ABOUT THE DECISION:

If freeze-all, the cycle ends here. The transfer will be a separate cycle, often a "frozen embryo transfer" or FET. Use a fresh planner for the FET.

If fresh transfer:

TRANSFER DATE: _____

TIME: _____

EMBRYO TRANSFERRED (clinic's description):

NUMBER REMAINING (frozen): _____

WHO WAS WITH ME: _____

WHAT THE CLINIC TOLD ME TO DO AFTER:

LUTEAL SUPPORT MEDICATIONS (by purpose, your clinic told you):

NEXT APPOINTMENT (often the beta test):

ONE LINE I WANT TO REMEMBER ABOUT TODAY:

The two-week wait, day 1 to 4

The hardest part of IVF, many people say. The clinic is no longer giving you daily numbers. Your body is doing whatever it is doing. You are waiting.

DAY 1 of the wait DATE: _____

Mood (one word): _____

Energy (1-5): _____

Physical signs (none required, just noting):

One line about today:

DAY 2 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line:

DAY 3 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line:

DAY 4 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line:

A NOTE FOR THIS WEEK:

The internet will tell you which symptoms mean what. The internet is wrong about you, specifically. Your body is doing something on its own timeline, and no symptom in the first week of the wait reliably predicts the outcome.

You are allowed to test at home if you want. You are also allowed to wait for the clinic's beta. Both are fine.

The two-week wait, day 5 to 8

DAY 5 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

DAY 6 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

DAY 7 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

DAY 8 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

The two-week wait, day 9 to 12

DAY 9 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

DAY 10 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

DAY 11 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

DAY 12 of the wait DATE: _____

Mood: _____

Energy (1-5): _____

One line: _____

The two-week wait, day 13 to 14

The end of the wait. The beta is tomorrow or today.

DAY 13 DATE: _____

Mood: _____

What I want to do today to take care of myself:

DAY 14 DATE: _____

The beta is scheduled for: _____

Who is going with me: _____

The first thing I will do after the call comes:

Beta test day

DATE: _____

TIME OF BLOOD DRAW: _____

TIME THE CLINIC CALLED: _____

THE BETA NUMBER: _____

WHAT THE CLINIC TOLD ME ABOUT THAT NUMBER:

NEXT STEP they recommended:

- ☐ Repeat beta in 48 hours
- ☐ Continue luteal support medications
- ☐ Schedule first ultrasound
- ☐ Stop medications
- ☐ Other:

THE FIRST PERSON I TOLD: _____

WHAT I WROTE THEM:

If they ask for a second beta:

SECOND BETA DATE: _____

NUMBER: _____

DOUBLING RATE per clinic: _____

WHAT THEY SAID:

Running cost tally

Updated as the cycle progresses. Keep receipts where you can.

EXPENSE	AMOUNT	DATE PAID
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____

RUNNING TOTAL: \$ _____

WHAT INSURANCE HAS COVERED: \$ _____

WHAT I HAVE PAID OUT OF POCKET: \$ _____

ANY UNEXPECTED COSTS:

FOR TAX TIME (if applicable):

Medical expenses related to IVF may be deductible above a certain threshold. Keep the receipts in one folder. Mark this date as the start of the folder.

If it worked

A positive beta is the start of a different chapter. The cycle is now an early pregnancy. You will move from the clinic team to (eventually) an OB. The relief and the fear may live in the same body.

THE FIRST THING I WANT TO REMEMBER:

THE FIRST PERSON I WANT TO TELL:

WHEN I AM PLANNING TO TELL THEM:

WHAT I AM SCARED OF, RIGHT NOW:

WHO I CAN TELL THAT TO:

THE NEXT APPOINTMENT IS:

Date: _____

Time: _____

With: _____

Bring: _____

A NOTE TO MYSELF:

A positive beta is a beginning. Early pregnancy is its own thing. The fear of loss is real. Let it be there without it owning you.

If you find yourself searching for symptoms or checking for blood every hour, that is anxiety asking for help. The doula, the therapist, the support group, the friend who has been through this, the perinatal mental-health line, all of those are open to you.

If it didn't

The negative beta is one of the hardest sentences in modern medicine. The clinic says a number. The number does not become a baby. The body has done what it did. You did not do anything wrong.

WHAT I AM FEELING RIGHT NOW:

ONE WORD FOR IT:

THE FIRST PERSON I TOLD, AND WHAT I SAID:

WHAT I WANT FROM THE PEOPLE WHO LOVE ME RIGHT NOW:

- ☐ Quiet company
- ☐ A meal dropped off
- ☐ Space, no contact
- ☐ A walk
- ☐ A phone call
- ☐ Just an acknowledgment, no fixing

WHAT I DO NOT WANT:

- ☐ Advice
- ☐ Statistics about the next try
- ☐ Stories about so-and-so who had to do five cycles
- ☐ The phrase "everything happens for a reason"
- ☐ The phrase "at least"
- ☐ Anything that minimizes what just happened

THE NEXT APPOINTMENT WITH THE CLINIC:

Date: _____

What they want to discuss:

WHAT I WANT TO ASK AT THAT APPOINTMENT:

1. _____
2. _____
3. _____

A PERMISSION SLIP FOR THE NEXT WEEK:

You are allowed to take time off if you can.

You are allowed to say no to the baby shower.

You are allowed to be angry.
You are allowed to not have a "next steps" plan yet.
You are allowed to grieve.
The cycle that did not work is still a loss.

If grief is loud and is not letting up, the perinatal mental-health line, your provider, or a therapist who specializes in fertility loss is the next call. P20 Pregnancy Loss Gentle Companion (Soothemade) is also there. We hope you do not need it. We made it because some people will.

Partner page

This page is for the support person.

The IVF cycle is happening in one body. The weight of it is in two lives.

What the cycle asks of the support person:

- Show up for the injections. Be the one who gives them, if you can. The trigger shot especially is a precise time. Setting an alarm together helps.
- Drive to retrieval. Pick up from retrieval. The anesthesia means the person cannot drive after.
- Manage the calls and texts during the two-week wait. The person doing the cycle is exhausted of telling.
- Track the costs alongside. The financial weight should not live in only one head.
- Watch your person. The mood swings of the medications are not character. The grief of a negative result is not a phase. The fear of a positive result is not ingratitude.

Things to say:

- "What do you need that you have not asked for."
- "I will handle this one. You sit."
- "I am with you, whichever way this goes."

Things not to say:

- "Just relax." (No.)
- "If this doesn't work, we can try again." (Not right now.)
- "Don't get your hopes up." (They are already up.)
- "At least we have each other." (True, but not in this moment.)

If your person says they cannot do another cycle:

Believe them. The decision to stop is yours to make together. There is no failure in stopping. There is no obligation to continue.

Telling people, or not telling people

There is no rule about who to tell during IVF. Some people tell everyone. Some people tell no one. Many tell a small list and protect the rest.

PEOPLE I HAVE TOLD:

1. _____
2. _____
3. _____
4. _____
5. _____

WHAT I TOLD THEM, ROUGHLY:

PEOPLE I HAVE NOT TOLD, BY CHOICE:

WHY I HAVE NOT TOLD THEM:

Scripts for the people who ask, if you do not want to share:

- "We are figuring some things out, and I will tell you more when I have something to tell."
- "We're in a season I'm not talking about much. Can we talk about [other topic]?"
- "I appreciate the question and I'm not ready to share. I will let you know when I am."

You do not owe anyone a fertility update.

What I am angry about today

A page just for the anger. Not the “we should be grateful” version of the cycle. The honest version.

What I am angry about today:

Who said the wrong thing this week:

What I wish I had said back:

What I wish the world understood about this:

A final letter

You did something extraordinary, and you did it by hand. The injections at exact times. The bloodwork. The waiting. The hoping with one part of yourself while another part stayed quiet, so you could survive the result either way.

If this cycle worked, you have begun. The fear in early pregnancy is real. The other Soothemade products will be there for the next chapters.

If this cycle did not work, you are not failing. You did the work of the cycle. The result was not yours to decide. The clinic, your provider, your partner if you have one, your therapist, your friends who have walked this, all of them are still here. You are allowed to be wrecked and you are allowed to be quiet and you are allowed to need a season.

If you are between cycles, sitting in the room where the decision lives, you do not have to know what comes next today.

The planner runs out at the beta. The rest of your life does not. Whatever the number was, you carried this cycle. That was the work.

Soothemade



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

notes.soothemade.com

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